Fax cover sheet

Mesa Verde General Hospital (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Danielle Okafor, RN CM
Phone: 555-0176
Date: 10/13/2026 10:00
Pages (including cover): 13
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

67M heart failure, cardiac stable. Independent with ADLs.
Looking for d/c 10/14 to recuperative care. Please review.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

ILLUMINATION Recuperative Care Referral Form

HEALTH + HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Mesa Verde General Hospital (fictional)
Dept/Floor: 4 Cardiac Step-Down
SW/CM/RN: Danielle Okafor, RN CM
Phone/Pager #: 555-0176
Email: dokafor@mesaverde.example
Nursing Station #: 555-0177
Authorized by: Dr. A. Haddad
Phone #:

Patient Information

Patient Name: Walter Doe MRN: MV-7731094 Date of Birth (dd/mm/yyy): 11/21/1958
Gender: ☒ Male ☐ Female ☐ Transgender ☐ Other Ethnicity: Black/African American
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
Heart failure exacerbation
Admit Date: 10/09/2026 Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 10/09/2026 Results: CXR - no TB findings

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes* ☒ No	Active Infection	Yes* ☒ No
*Explain		*Explain	
Self Administer All Meds	☒ Yes No*	Ambulatory	☒ Yes No
Continent of Bowel & Bladder	☒ Yes No	Assistive Device	Yes* ☒ No
Colostomy/Ileostomy	Yes ☒ No	*Explain	
Foley Catheter	Yes ☒ No	Hospice	Yes* ☒ No
Diabetes	Yes ☒ No	*Explain	
Insulin	Yes ☒ No	Car	Yes ☒ No
Diabetes related Oral Meds	Yes ☒ No	Service Animal	Yes ☒ No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? ☒ No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain

Length of Stay Authorized in Recupertive Care: 21 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)

Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Walter Doe

DOB: 11/21/1958

Member ID: OCH-62049981

Referring Provider

Facility: Mesa Verde General Hospital (fictional)

Clinical

Primary Diagnosis: Acute on chronic systolic heart failure

ICD-10: I50.23

Member is experiencing homelessness [X]

Member at risk of hospitalization/re-hospitalization [X]

Requested Start Date: 10/14/2026

Requested Days: 21

Admit Date: 10/09/2026

Plan Use Only

Status: Pending review

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Patient face sheet

Mesa Verde General Hospital (fictional) - PATIENT FACE SHEET

Patient: Walter Doe
MRN: MV-7731094
Account #: 99731094
DOB: 11/21/1958
Sex: Male
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 10/09/2026 20:10 (ED arrival 10/09/2026 14:35)
Admit Type: Inpatient
Unit/Bed: 4CSD-407
Attending: Amira Haddad, MD (Cardiology)

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

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History and Physical - page 1

HISTORY AND PHYSICAL

Date: 10/09/2026

CC

Shortness of breath, unable to lie flat

HPI

67 y/o M, unhoused x 4 yrs (sleeps at Example Park), with nonischemic cardiomyopathy (HFrEF), HTN and CKD 3b, presents with 10 days of worsening dyspnea, orthopnea, 12 lb weight gain and abdominal swelling. Ran out of meds 1 month ago. Third HF admission in 6 months.

PMH

- Nonischemic cardiomyopathy, HFrEF (LVEF 30% on outside echo 2025)
- Hypertension
- CKD stage 3b
- No history of MI. Cath 2024: no obstructive CAD (per pt)

Meds PTA

- Furosemide 40 mg BID, carvedilol 6.25 mg BID, sacubitril-valsartan 24/26 mg BID, spironolactone 25 mg daily (none taken x 1 month)

Allergies

NKDA

Social

Former smoker. EtOH none x 5 yrs. Denies drug use.

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Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

History and Physical - page 2

Exam

T 97.8F HR 112 BP 186/104 RR 26 SpO2 88% RA -> 94% 4L NC
JVD to ear, S3 gallop, bibasilar crackles to mid-lung fields, 3+ edema to thighs, ascites
AOx4

Assessment/Plan

1. Acute on chronic systolic HF, decompensated - furosemide 80 mg IV BID, nitroglycerin infusion for afterload/BP, strict I/O, daily weights
2. Hypertensive urgency - IV nitroglycerin, add hydralazine
3. CKD 3b - monitor Cr with diuresis
4. TTE, cardiology consult

Signed: Amira Haddad, MD 10/09/2026 21:30

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Echocardiogram report

TRANSTHORACIC ECHOCARDIOGRAM REPORT

Date: 10/10/2026

Indication: decompensated heart failure

Findings

Left ventricle: severely dilated (LVIDd 7.1 cm). Severe global hypokinesis.

LVEF (biplane Simpson): 15%

Right ventricle: moderately reduced function, TAPSE 12 mm

Moderate-severe functional mitral regurgitation

Estimated RVSP 58 mmHg

Small pericardial effusion, no tamponade

Conclusion

Severely reduced LV systolic function, LVEF 15%. Decline from LVEF 30% (outside echo 2025).

Read by: S. Lindgren, MD (Cardiology)

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Cardiology progress note

CARDIOLOGY PROGRESS NOTE - HD#4

Date: 10/13/2026 08:30

S: Breathing better but still orthopneic (2 pillows). Uop 2.1 L/24h.

O: T 98.0F HR 96 BP 158/92 RR 20 SpO2 94% RA. Wt 88.2 kg (admit 93.6 kg). JVD 12 cm, crackles bases, 2+ edema to knees. AOx4.

Cr 2.1 (admit 1.8).

A/P:

1. Acute on chronic HFrEF, LVEF 15% (new decline from 30%) - remains volume overloaded. Furosemide infusion 15 mg/hr continues (started 10/11 after inadequate response to IV bolus). Not yet ready to transition to PO diuretic.
2. HTN - nitroglycerin infusion weaned off 10/12 16:00. Hydralazine 10 mg IV x1 given 10/13 02:10 for SBP 184. Uptitrating PO hydralazine/isosorbide dinitrate.
3. Advanced HF team consult for LVEF 15% - evaluation for advanced therapies; GDMT limited by Cr.
4. Dispo - NOT medically ready for discharge. Anticipate at least 3-5 more days.

Signed: Amira Haddad, MD (Cardiology)

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Medication administration record (excerpt)

MEDICATION ADMINISTRATION RECORD - IV MEDICATIONS

Patient: Walter Doe MRN: MV-7731094

Furosemide 80 mg IV BID | 10/09 22:00 - 10/11 10:00 | DISCONTINUED 10/11 (changed to infusion)

Furosemide infusion 10-15 mg/hr IV continuous | Start 10/11 14:00 | Rate increased to 15 mg/hr 10/12 08:00 | RUNNING as of 10/13 08:00

Nitroglycerin infusion 10-80 mcg/min IV | Start 10/09 21:45 | Weaned off 10/12 16:00

Hydralazine 10 mg IV PRN SBP > 180 | Given 10/13 02:10 (SBP 184)

PO

Hydralazine 25 mg PO TID | Start 10/12 18:00

Isosorbide dinitrate 20 mg PO TID | Start 10/12 18:00

Carvedilol - HELD (low output concern per cardiology)

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Nursing shift note

NURSING SHIFT NOTE

Date: 10/13/2026 06:45

Pt AOx4, calm, cooperative. Independent with ADLs at bedside, ambulating to bathroom with IV pole (furosemide drip infusing at 15 mg/hr).

02:00 SBP 184/102, MD notified, hydralazine 10 mg IV given 02:10, recheck 03:00 SBP 152/90.

Telemetry: sinus tachycardia 100–110, runs of NSVT x2 (5 beats) overnight, MD aware.

Uop 2.1 L/24h via urinal. Daily wt 88.2 kg.

Signed: P. Nwosu, RN

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ECG report

12-LEAD ECG REPORT

Date/Time: 10/09/2026 14:50
Rate 112 bpm. Rhythm: sinus tachycardia
PR 182 ms, QRS 134 ms (LBBB), QTc 488 ms
Left bundle branch block, unchanged from 2025 outside tracing per pt records
No acute ST elevation
Read by: S. Lindgren, MD (Cardiology)

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Labs and vitals

LABS

10/09 | BNP 3,980 H | Na 131 L | K 4.4 | Cr 1.8 H | Hgb 11.4 L

Troponin I (hs): 10/09 15:00 12 ng/L | 10/09 18:00 13 ng/L | 10/10 06:00 11 ng/L (ref <17; negative x3)

10/12 | Na 133 L | K 3.8 | Cr 2.0 H

10/13 | Na 132 L | K 3.7 | Cr 2.1 H | Mg 1.9

10/09 MRSA nares PCR: Negative

10/13 06:00 SARS-CoV-2 rapid antigen: Negative

10/09 CXR: cardiomegaly, pulmonary edema, bilat pleural effusions

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
10/12 08:00	98.1	104	168/96	22	93%	RA
10/12 16:00	98.3	102	162/94	20	93%	RA
10/13 02:00	98.0	108	184/102	22	92%	RA
10/13 03:00	98.0	100	152/90	20	93%	RA
10/13 08:30	98.0	96	158/92	20	94%	RA

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Medication list

CURRENT MEDICATIONS

Patient: Walter Doe MRN: MV-7731094

IV

- Furosemide infusion 15 mg/hr IV continuous
- Hydralazine 10 mg IV PRN SBP > 180

PO

- Hydralazine 25 mg PO TID
- Isosorbide dinitrate 20 mg PO TID
- Spironolactone 25 mg PO daily
- Potassium chloride 20 mEq PO daily

Held

- Carvedilol, sacubitril-valsartan (held for Cr / low output)

Discharge medication list: not yet prepared

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